

Clinical Study Summary Report (CSSR)

Document Status: Draft | Version: 0.1 | Date: 02-Apr-2026

1. Administrative & Study Identification

Full Study Title: General Use-Results Study of Calquence Capsules 100 mg in Patients With Previously Untreated Chronic Lymphocytic Leukaemia (Including Small Lymphocytic Lymphoma) **Short Title/Acronym:** Calquence CLL 1L Japan PMS Study **Protocol Number:** D822EC00001 **NCT Number:** NCT05665374 **Sponsor Name:** AstraZeneca **Investigational Product:** Calquence capsules 100 mg (acalabrutinib) **Phase of Development:** Phase IV (Post-Marketing Surveillance / Observational Study) **Medical Monitor:** AstraZeneca Clinical Operations Lead (Japan)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To understand the incidence of adverse drug reactions (ADRs) of Calquence capsules 100 mg (acalabrutinib) used in patients with previously untreated chronic lymphocytic leukaemia (CLL) (including small lymphocytic lymphoma (SLL)) in the post-marketing setting under actual use. **Secondary Objectives:** To evaluate the general safety, tolerability, and real-world effectiveness of first-line acalabrutinib in the Japanese patient population.

2.2 Background & Rationale This investigation is conducted as an additional pharmacovigilance activity specified in the Japan Risk Management Plan (J-RMP) of CALQUENCE. It is performed in compliance with the Ministerial Ordinance on Good Post-marketing Study Practice (GPSP Ordinance) for the purpose of application for re-examination under Article 14-4 of the Law for Ensuring Quality, Efficacy, and Safety of Drugs and Medical Devices (PMD Act) in Japan.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Post-Marketing Surveillance) **Control Type:** Single-arm (Cohort Study) **Blinding:** Open-label / Non-blinded **Randomization:** N/A (Non-Probability Sample)

3.2 Planned Enrollment & Duration Target **NS:** 67 patients **Number of Sites:** Multiple sites in Japan (e.g., Kyoto, Miyazaki) **Estimated Study Start:** 19-Jun-2023 (Actual) **Estimated Primary Completion:** 20-Aug-2025 (Actual)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Patients with previously untreated chronic lymphocytic leukaemia (including small lymphocytic lymphoma). 2. Patients who are treated with Calquence for the first time. 3. Patients enrolled in the study after the approval date of the supplemental Japanese New Drug Application (sJNDA) for the additional indication.

4.2 Exclusion Criteria 1. None specified.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Calquence (acalabrutinib) capsules 100 mg, prescribed per standard local label dosing for CLL/SLL. **Route of Administration:** Oral **Duration of Treatment:** Evaluated over a 24-week primary outcome timeframe; treatment duration aligns with real-world clinical practice.

5.2 Concomitant & Prohibited Therapy Permitted: Standard concomitant medications for comorbidities according to the treating physician's discretion and Japanese package insert. **Prohibited:** Contraindicated drugs per the local prescribing information for Calquence (e.g., strong CYP3A inhibitors/inducers, dependent on real-world management guidelines).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening / Baseline	Day 0 Diagnosis confirmation, routine baseline labs, first prescription of Calquence
Interim Follow-ups	Per standard practice	Routine clinical visits, safety monitoring, ADR tracking (focus on infections, cytopenias)	End of Observation
Week 24	Collection of ADR incidence data, general safety evaluation		

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence of ADRs related to Calquence Safety Specifications, specifically focusing on infection and bone marrow depression over a 24-week period. **Measurement Method:** Clinical assessment and routine laboratory tracking submitted by treating physicians via post-marketing surveillance forms.

7.2 Secondary & Exploratory Endpoints Collection of real-world data regarding unlisted adverse events, patient tolerability, and treatment continuation/discontinuation rates in a generalized clinical setting.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Conducted via observational reporting by healthcare providers under actual use conditions.

Serious Adverse Events (SAEs): Required to be reported in accordance with Japanese GPSP and PMD Act regulations. **Data**

Monitoring Committee (DMC): N/A (Standard local pharmacovigilance tracking applies).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all enrolled patients who received at least one dose of Calquence and have follow-up data).

Sample Size Justification: A target of 67 patients is deemed sufficient to capture the incidence of targeted ADRs (infection, bone marrow depression) in the newly approved indication cohort within Japan. **Handling of Missing Data:** As an observational real-

world study, data will generally be evaluated using descriptive statistics with no formal imputation for missing values unless specified by the J-RMP framework.

10. Quality Assurance & Regulatory Compliance

GCP/GPSP Compliance: This study is conducted in strict accordance with the Ministerial Ordinance on Good Post-marketing Study Practice (GPSP Ordinance) in Japan. **Monitoring Plan:** Routine collection and verification of surveillance forms submitted by participating sites. **Audit Trail:** Handled via AstraZeneca's validated databases for regulatory submissions to the Japanese Pharmaceuticals and Medical Devices Agency (PMDA) for re-examination under Article 14-4 of the PMD Act.

11. Study Identifiers & Governance

Primary ID: D822EC00001 **Registry Number:** NCT05665374 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** Calquence CLL 1L Japan PMS Study **Official Regulatory Title:** General Use-Results Study of Calquence Capsules 100 mg in Patients With Previously Untreated Chronic Lymphocytic Leukaemia (Including Small Lymphocytic Lymphoma)

12. Clinical Framework (CLL / SLL Specific)

Primary Condition: Chronic Lymphocytic Leukaemia (CLL) and Small Lymphocytic Lymphoma (SLL) **Diagnosis Threshold:** Confirmed diagnosis requiring first-line (1L) systemic therapy **Medical**

Methodology: Targeted Oral Therapy (Bruton's Tyrosine Kinase

[BTK] inhibitor) **Optimization Target:** Real-world safety tracking and adverse event risk management

13. Study Procedures & Methodology

Management Pathway: Standard real-world clinical pathway for CLL/SLL management in Japan. **Education Protocol (HCP):** Distribution of the Japan Risk Management Plan (J-RMP) safety guidance for acalabrutinib. **Education Protocol (Patient):** Routine counseling on potential ADRs, specifically signs of infection and bleeding. **Quality Audit Mechanism:** Regulatory-level pharmacovigilance tracking and GPSP documentation audits.

14. Observation & Follow-Up Schedule

Total Duration: 24 weeks for primary ADR tracking **Onsite Visit Cadence:** Determined by the treating physician per routine care. **Remote Monitoring Frequency:** Routine data entry by the participating Japanese sites. **Checklist Review Interval:** Data consolidation for the PMDA re-examination periodic reports.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator / PMS Lead	[Name]	_____	[DD-MMM-YYYY]	Clinical Operations Lead (Japan)	[Name]	_____	[DD-MMM-YYYY]
Pharmacovigilance Lead	[Name]	_____	[DD-MMM-YYYY]				